LABORATORY REPORT

Redcliffe labs

Patient NAME	**: Ms Divya**		
DOB/Age/Gender	: 38 Y/Female	Report STATUS	: Final Report
Patient ID / UHID	: 16746904/RCL16605898	Barcode NO	: 30847132
Referred BY	: Self	Sample Type	: Whole blood EDTA
Sample Collected	: May 14, 2026, 09:48 AM	Report Date	: May 14, 2026, 01:25 PM.

NABL-M(EL)T-03192
MEDICAL ENTRY LEVEL TESTING LABORATORY RECOGNITION PROGRAM

21C01 - 02 HEALTHI

Complete Blood Count (CBC)

Test Description	Value(s)	Unit(s)	Reference Range
RBC Parameters			
Hemoglobin *Cyanide-free Spectrophotometry*	**9.1 L***	g/dL	12.0 - 15.0
RBC Count *Electrical impedance*	4.4	10^6/µl	3.8 - 4.8
PCV *Calculated*	**28.6 L***	%	36 - 46
MCV *Direct Measure Impendence*	**65.5 L***	fl	83 - 101
MCH *Calculated*	**20.9 L***	pg	27 - 32
MCHC *Calculated*	31.9	g/dL	31.5 - 34.5
RDW (CV) * *Calculated*	14	%	11.6 - 14.0
RDW-SD * *Calculated*	**32.8 L***	fl	35.1 - 43.9
WBC Parameters			
TLC *Electrical impedance and microscopy*	7.1	10^3/µl	4 - 10
Differential Leucocyte Count			
Neutrophils *Flow cytometry*	59.7	%	40-80
Lymphocytes *Flow cytometry*	32.9	%	20-40
Monocytes *Flow cytometry*	4.7	%	2-10
Eosinophils *Flow cytometry*	1.7	%	1-6
Basophils *Flow cytometry*	1	%	<2
Absolute Leukocyte Counts *			
Neutrophils. *	4.24	10^3/µl	2 - 7
Lymphocytes. *	2.34	10^3/µl	1 - 3
Monocytes. *	0.33	10^3/µl	0.2 - 1.0
Eosinophils. *	0.12	10^3/µl	0.02 - 0.5
Basophils. *	0.07	10^3/µl	0.02 - 0.5
Platelet Parameters			
Platelet Count *Electrical impedance and microscopy*	305	10^3/µl	150 - 410

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)
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Dr. Tejal K Bhatt
MBBS, MD (Path)
Consultant Pathologist

Booking Centre :- Svasth Noida, .
Processing Lab :- Redcliffe lifetech private limited, Shop no-1a, Ground floor, Zenon, near Unique hospital, Ring road, Surat, Gujarat-395002.

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Test Description	Value(s)	Unit(s)	Reference Range
Mean Platelet Volume (MPV) * *Calculated*	**9 L***	fL	9.3 - 12.1
PCT * *Calculated*	0.3	%	0.17 - 0.32
PDW * *Calculated*	13.4	fL	8.3 - 25.0
P-LCR * *Calculated*	24.1	%	18 - 50
P-LCC * *Calculated*	73	10^9/L	44 - 140
Mentzer Index * *Calculated*	14.89	%	> 13

ADVICE:

SERUM IRON PROFILE

STOOL FOR OCCULT BLOOD

CHECK FOR EXCESSIVE MENSTRUATION HISTORY.

Interpretation:
CBC provides information about red cells, white cells and platelets. Results are useful in the diagnosis of anemia, infections, leukemias, clotting disorders and many other medical conditions.

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Test Description	Value(s)	Unit(s)	Reference Range

Erythrocyte Sedimentation Rate (ESR)

Test Description	Value(s)	Unit(s)	Reference Range
ESR - Erythrocyte Sedimentation Rate *MODIFIED WESTERGREN*	**30 H***	mm/hr	0 - 12

Interpretation:
ESR is also known as Erythrocyte Sedimentation Rate. An ESR test is used to assess inflammation in the body. Many conditions can cause an abnormal ESR, so an ESR test is typically used with other tests to diagnose and monitor different diseases. An elevated ESR may occur in inflammatory conditions including infection, rheumatoid arthritis ,systemic vasculitis, anemia, multiple myeloma , etc. Low levels are typically seen in congestive heart failure, polycythemia ,sickle cell anemia, hypo fibrinogenemia , etc.

Reference- Dacie and lewis practical hematology

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Referred BY	: Self	Sample Type	: Whole blood EDTA
Sample Collected	: May 14, 2026, 09:48 AM	Report Date	: May 14, 2026, 12:24 PM.

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MEDICAL ENTRY LEVEL TESTING LABORATORY RECOGNITION PROGRAM

Test Description	Value(s)	Unit(s)	Reference Range

HbA1C (Glycosylated Haemoglobin)

Glycosylated Hemoglobin (HbA1c) *N- terminal fructosyl dipeptides*	**6 H***	%	<5.7
Estimated Average Glucose * *Calculated*	125.5	mg/dL	Refer Table Below

Interpretation:
Interpretation For HbA1c% As per American Diabetes Association (ADA)

Reference Group	HbA1c in %
Non diabetic adults >=18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5
Therapeutic goals for glycemic control	Age > 19 years Goal of therapy: < 7.0 Age < 19 years Goal of therapy: <7.5

Note:
1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled.
2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.

Comments :
HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations ADA criteria for correlation between HbA1c & Mean plasma glucose levels.

HbA1c(%)	**Mean Plasma Glucose (mg/dL)**	**HbA1c(%)**	**Mean Plasma Glucose (mg/dL)**
6	126	12	298
8	183	14	355
10	240	16	413

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Patient ID / UHID	: 16746904/RCL16605898	Barcode NO	: 31493856
Referred BY	: Self	Sample Type	: FLUORIDE F
Sample Collected	: May 14, 2026, 09:48 AM	Report Date	: May 14, 2026, 12:13 PM.

NABL-M(EL)T-03192

MEDICAL ENTRY LEVEL TESTING LABORATORY RECOGNITION PROGRAM

Test Description	Value(s)	Unit(s)	Reference Range

Blood Sugar Fasting

Test Description	Value(s)	Unit(s)	Reference Range
Glucose Fasting *Hexokinase*	85	mg/dL	70 - 100

Interpretation:

Status	Fasting plasma glucose in mg/dL
Normal	70 - 100
Impaired fasting glucose	101 - 125
Diabetes	≥126

Reference : American Diabetes Association

Comment :
Blood glucose determinations in commonly used as an aid in the diagnosis and treatment of diabetes. Elevated glucose levels (hyperglycemia) may also occur with pancreatic neoplasm, hyperthyroidism, and adrenal cortical hyper function as well as other disorders. Decreased glucose levels (hypoglycemia) may result from excessive insulin therapy insulinoma, or various liver diseases.

Note
1.The diagnosis of Diabetes requires a fasting plasma glucose of > or = 126 mg/dL or a random / 2 hour plasma glucose value of > or = 200 mg/dL with symptoms of diabetes mellitus.
2.Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis.

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Referred BY	: Self	Sample Type	: Serum
Sample Collected	: May 14, 2026, 09:48 AM	Report Date	: May 14, 2026, 12:59 PM.

NABL-M(EL)T-03192

MEDICAL ENTRY LEVEL TESTING LABORATORY RECOGNITION PROGRAM

Test Description	Value(s)	Unit(s)	Reference Range

Blood Urea

Test Description	Value(s)	Unit(s)	Reference Range
Blood Urea *Urease*	24	mg/dL	19 - 44.1

Interpretation:
Urea is a renal function test that is often interpreted with creatinine. It is also useful when measured before and after dialysis treatments.

Creatinine, Serum

Test Description	Value(s)	Unit(s)	Reference Range
Creatinine *Kinetic Alkaline Picrate*	0.75	mg/dL	0.72 - 1.25
eGFR (CKD-EPI) *	104.43	ml/min/1.73 sq m	Normal Or High: >= 90 Mild Or Decrease: 60-89 Mild To Moderate Decrease: 45-59 Mild To Severe Decrease: 30-44 Severe Decrease: 15-29 Kidney Failure: < 15

Interpretation:
Creatinine estimation is done to assess kidney function. It is not dependent on dietary factors. Normal values are obtained in kidney diseases, except in advanced renal failure and therefore its estimation is more valuable if coupled with clearance.

"eGFR test is applicable for patients aged 18 years or more."

Uric Acid

Test Description	Value(s)	Unit(s)	Reference Range
Uric Acid *Uricase*	4.3	mg/dL	2.6 - 6.0

Interpretation:
Serum uric acid levels are very labile and show day to day and seasonal variation in some people. Levels are also increased by emotional stress, total fasting and increased body weight. Serum uric acid levels are used to diagnose and monitor treatment of gout, monitor chemotherapeutic treatment of neoplasms to avoid renal urate deposition with possible renal failure.

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NABL-M(EL)T-03192

MEDICAL ENTRY LEVEL TESTING LABORATORY RECOGNITION PROGRAM

Liver Function Test (LFT)

Test Description	Value(s)	Unit(s)	Reference Range
Bilirubin Total *Diazonium Salt*	0.4	mg/dL	0.2 - 1.2
Bilirubin Direct *Diazo Reaction*	0.2	mg/dL	0.0 - 0.5
Bilirubin Indirect * *Calculation (T Bil - D Bil)*	0.2	mg/dL	0.1 - 1.0
SGOT/AST *NADH without P5P*	17	U/L	5 - 34
SGPT/ALT *NADH without P5P*	11	U/L	0 to 55
SGOT/SGPT Ratio * *Calculated*	1.55	-	-
Alkaline Phosphatase *Para-Nitrophenyl Phosphate*	63	U/L	40 - 150
Total Protein *Biuret*	7	g/dL	6.4 - 8.3
Albumin *BCG*	4.22	gm/dL	3.8 - 5.0
Globulin * *Calculation (T.P - Albumin)*	2.78	g/dL	2.3 - 3.5
Albumin :Globulin Ratio * *Calculation (Albumin/Globulin)*	1.52	-	1.0 - 2.1
Gamma Glutamyl Transferase (GGT) * *L-Gamma-Glutamyl-3-Carboxy-4-Nitroanilide Substra-*	14	U/L	12 - 64

Interpretation:
The liver filters blood, metabolizes nutrients, detoxifies harmful substances, and produces blood clotting proteins. Liver cells contain enzymes that facilitate these functions. When cells are damaged, enzymes leak into the blood, detectable through blood tests.

Key enzymes tested:
1. AST (SGOT): may indicate tissue injury / damage in muscles or liver.
2. ALT (SGPT): Primarily in the liver. Elevated ALT and AST suggest liver damage.
3. Alkaline Phosphatase & GGT: Linked to bile production and flow. Elevated levels may indicate bile flow issues related to the liver, gallbladder, or bile ducts.

Blood proteins, **albumin and globulin**, are essential for growth, development, and health.
1. Low protein: May indicate bleeding, liver disorders, malnutrition, or agammaglobulinemia.
2. High protein (Hyperproteinemia): Often due to dehydration or increased protein production.
3. Low albumin: Caused by poor diet, kidney, or liver disease.
4. High albumin: Usually due to severe dehydration.

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NABL-M(EL)T-03192 MEDICAL ENTRY LEVEL TESTING LABORATORY RECOGNITION PROGRAM

Lipid Profile

Test Description	Value(s)	Unit(s)	Reference Range
Total Cholesterol *Enzymatic - Cholesterol Oxidase*	199	mg/dL	<200
Triglycerides *Glycerol Phosphate Oxidase*	78	mg/dL	<150
HDL Cholesterol *CHOD/CHER*	51	mg/dL	>40
Non HDL Cholesterol * *Calculated*	**148 H***	mg/dL	<130
LDL Cholesterol * *Calculated*	**132.4 H***	mg/dL	<100
V.L.D.L Cholesterol * *Calculated*	15.6	mg/dL	< 30
Chol/HDL Ratio * *Calculated*	3.9	Ratio	3.0 - 5.0
HDL/ LDL Ratio * *Calculated*	**0.39 L***	Ratio	0.5 - 3.0
LDL/HDL Ratio * *Calculated*	2.6	-	-

Interpretation:
Lipid level assessments must be made following 10 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. NCEP recommends of 3 different samples to be drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single assays. Intraindividual (within-person) variation in triglyceride (TG) levels is high, often showing a 12.9% to 40.8% variation in healthy individuals within 1 year,. This variability is driven by a combination of biological, lifestyle, and physiological factors that fluctuate rather than remaining constant.
Causes of variation include

- Diet: Levels spike 5–10 times after meals.Unhealthy high saturated fat diets like non veg diet sweetened beverages , fried or processed foods , and alcohol intake can significantly raise triglyceride values.
- Lifestyle: Obesity , sedentary lifestyle, and smoking can raise levels.
- Biology: Pregnancy (estrogen), aging, and conditions like Diabetes or Metabolic Syndrome cause major shifts.
- Drugs like : Beta-blockers, steroids, tamoxifen, thiazides, and oral contraceptives can raise triglyceride levels

National Lipid Association Recommendations (NLA-2014)	Total Cholesterol (mg/dL)	Triglyceride (mg/dL)	LDL Cholesterol (mg/dL)	Non HDL Cholesterol (mg/dL)
Optimal	<200	<150	<100	<130
Above Optimal			100-129	130 - 159
Borderline High	200-239	150-199	130-159	160 - 189
High	>=240	200-499	160-189	190 - 219
Very High	-	>=500	>=190	>=220

HDL Cholesterol	
Low	High
<40	>=60

Risk Stratification for ASCVD (Atherosclerotic Cardiovascular Disease) by Lipid Association of India.

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Test Description	Value(s)	Unit(s)	Reference Range

Risk Category	A. CAD with > 1 feature of high risk group
Extreme risk group	B. CAD with >1 feature of very high risk group of recurrent ACS (within 1 year) despite LDL-C <or = 50 mg/dl or poly vascular disease
Very High Risk	1.Established ASCVD 2.Diabetes with 2 major risk factors of evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
High Risk	1. Three major ASCVD risk factors 2. Diabetes with 1 major risk factor or no evidence of end organ damage 3. CHD stage 3B or 4. 4 LDL >190 mg/dl 5. Extreme of a single risk factor 6. Coronary Artery Calcium - CAC > 300 AU 7. Lipoprotein a >/= 50 mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age >/=45 years in Males & >/= 55 years in Females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by Lipid Association of India in 2020.

Risk Group	**Treatment Goals**		**Consider Drug Therapy**	
	LDL-C (mg/dl)	**Non-HDL (mg/dl)**	**LDL-C (mg/dl)**	**Non-HDL (mg/dl)**
Extreme Risk Group Category A	<50 (Optional goal <OR = 30)	<80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	>OR = 30	>OR = 60	> 30	> 60
Very High Risk	<50	<80	>OR = 50	>OR = 80
High Risk	<70	<100	>OR = 70	>OR = 100
Moderate Risk	<100	<130	>OR = 100	>OR = 130
Low Risk	<100	<130	>OR = 130*	>OR = 160

*** After an adequate non-pharmacological intervention for at least 3 months.**

References :

- Management of Dyslipidaemia for the Prevention of Stroke : Clinical practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology,2022,20,134-155.
- Nordestgaard, B. A Test in Context: Lipid Profile, Fasting Versus Nonfasting. JACC. 2017 Sep, 70 (13) 1637–1646.
- P.N.M. Demacker, R.W.B. Schade, R.T.P. Jansen, A. Van 't Laar, Intra-individual variation of serum cholesterol, triglycerides and high density lipoprotein cholesterol in normal humans, Atherosclerosis,
- Parhofer KG, Laufs U: The diagnosis and treatment of hypertriglyceridemia. Dtsch Arztebl Int 2019; 116: 825–32. DOI: 10.3238/arztebl.2019.0825

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Test Description	Value(s)	Unit(s)	Reference Range

Calcium, Serum

Test Description	Value(s)	Unit(s)	Reference Range
Calcium Serum *Arsenazo III*	9.3	mg/dL	8.4 - 10.2

Interpretation:
Elevated calcium value are associated with hyperparathyrodism, multiple myeloma, neoplasms of bone and parathyroid & conditions of rapid demineralization, tetany & occasionally with nephrosis & pancreatitis. Severe nephritis & uremia may cause either elevated or lowered calcium values. Decreased values of calcium are noted in hypoparathyroidism, vitamin D deficiency, renal insufficiency, hypoproteinemia, malabsorption syndrome, severe pancreatitis with pancreatic necrosis and pseudo-hypoparathyroidism.

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Vitamin B12 / Cyanocobalamin

Test Description	Value(s)	Unit(s)	Reference Range
Vitamin - B12 *CMIA*	200	pg/mL	187 - 883

Interpretation:
Low Values are a sign of a vitamin B12 deficiency. People with this deficiency are likely to have or develop symptoms.
Causes of vitamin B12 deficiency include:Not enough vitamin B12 in diet (rare except with a strict vegetarian diet), Diseases that cause malabsorption (for example, celiac disease and Crohn's disease), Lack of intrinsic factor, Above normal heat production (for example, with hyperthyroidism), Pregnancy. Increased vitamin B12 levels are uncommon. Usually excess vitamin B12 is removed in the urine. Conditions that can increase B12 levels include: Liver disease (such as cirrhosis or hepatitis), Myeloproliferative disorders (for example, polycythemia vera and chronic myelocytic leukemia).

Vitamin B12: Low Levels can cause malabsorption, Lack of intrinsic factor, Above normal heat production (for example, with hyperthyroidism), Pregnancy.High Level Liver disease, Myeloproliferative disorders (for example, polycythemia vera and chronic myelocytic leukemia).

1. Out of 140 healthy indian population, 91% of Vitamin B 12 concentrations was at lower level: 59.00 pg/ml and upper level: 700.00 pg/ml

"Patients on Biotin supplement may have interference in some immunoassays. Ref: Arch Pathol Lab Med—Vol 141, November 2017. With individuals taking high dose Biotin (more than 5 mg per day) supplements, at least 8-hour wait time before blood draw is recommended."

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Vitamin D 25 Hydroxy

Test Description	Value(s)	Unit(s)	Reference Range
Vitamin D 25 - Hydroxy *CMIA*	**24.5 L***	ng/mL	Deficiency : < 10 ng/mL Insufficient : 10-30 ng/mL Sufficient : 30-100 ng/mL Hypervitaminosis : > 100 ng/mL

Interpretation:
25-Hydroxy vitamin D represents the main body reservoir and transport form. Mild to moderate deficiency is associated with Osteoporosis / Secondary Hyperparathyroidism while severe deficiency causes Rickets in children and Osteomalacia in adults. Prevalence of Vitamin D deficiency is approximately >50% specially in the elderly. This assay is useful for diagnosis of vitamin D deficiency and Hypervitaminosis D. It is also used for differential diagnosis of causes of Rickets & Osteomalacia and for monitoring Vitamin D replacement therapy.

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Test Description	Value(s)	Unit(s)	Reference Range

Thyroid Profile Total

Triiodothyronine (T3) *CMIA*	88.8	ng/dL	35 - 193
Total Thyroxine (T4) *CMIA*	9.2	µg/dL	4.87 - 11.2
Thyroid Stimulating Hormone (Ultrasensitive) *CMIA*	1.8165	mIU/L	0.35 - 4.94

Interpretation:

Pregnancy	Refrence Range TSH
1st Trimester	0 .1 - 2.5
2nd Trimester	0.2 - 3.0
3rd Trimester	0.3 - 3.0

Note: TSH levels are subject to circadian variation, reaching peak levels between 2-4 am. and at a minimum between 6-10 pm. The variation is of 50 %, hence time of the day has influence on the measured serum TSH concentrations. Apart from nocturnal circadian surge other biological factors responsible for TSH variation include pulsatile secretion, along with seasonal changes and lifestyle factors such as smoking, BMI, and diet. Other contributors include aging, non-thyroidal illnesses, medication interference, and rare analytical issues like macro-TSH

Clinical Use:
1. Diagnose Hypothyroidism & Hyperthyroidism
2. Monitor T4 therapy
3. Measure subnormal TSH levels

Increased TSH: Primary hypothyroidism, Subclinical hypothyroidism, TSH-dependent hyperthyroidism, Thyroid hormone resistance
Decreased TSH: Graves' disease, Autonomous thyroid hormone secretion, TSH deficiency
Thyroid malfunction (hyper or hypo) affects T3 & T4 levels. Pituitary or hypothalamic issues also influence thyroid activity.

1. Primary Hypothyroidism: High TSH levels.
2. Secondary/Tertiary Hypothyroidism: Low TSH levels.
3. Euthyroid Sick Syndrome: Abnormal thyroid test results due to non-thyroidal illnesses (NTI).
TBG levels are stable in healthy individuals but may be altered by pregnancy, estrogens, androgens, steroids, or glucocorticoids, causing inaccurate T3 & T4 readings.

TSH	T4	T3	Interpretation
High	Normal	Normal	Mild (subclinical) hypothyroidism
High	Low	Low Or Nomral	Hypothyroidism
Low	Normal	Normal	Mild (subclinical) hyperthyroidism
Low	High Or Normal	High Or Normal	Hyperthyroidism
Low	Low Or Normal	Low Or Normal	Nonthyroidal illness; pituitary (secondary) hypothyroidism
Normal	High	High	Thyroid hormone resistance syndrome (a mutation in the thyroid hormone receptor decreases thyroid hormone function)

References

- Van der Spoel E, Roelfsema F and van Heemst D (2021) Within-Person Variation in Serum Thyrotropin Concentrations: Main Sources, Potential Underlying Biological Mechanisms, and Clinical Implications. Front. Endocrinol. 12:619568. doi: 10.3389/fendo.2021.619568

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.

Dr. Tejal K Bhatt
MBBS, MD (Path)
Consultant Pathologist

Patient NAME	**: Ms Divya**		
DOB/Age/Gender	: 38 Y/Female	Report STATUS	: Final Report
Patient ID / UHID	: 16746904/RCL16605898	Barcode NO	: 30855868
Referred BY	: Self	Sample Type	: Serum
Sample Collected	: May 14, 2026, 09:48 AM	Report Date	: May 14, 2026, 12:59 PM.

Test Description	Value(s)	Unit(s)	Reference Range

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.

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Patient NAME	**: Ms Divya**		
DOB/Age/Gender	: 38 Y/Female	Report STATUS	: Final Report
Patient ID / UHID	: 16746904/RCL16605898	Barcode NO	: 32276138
Referred BY	: Self	Sample Type	: Spot Urine
Sample Collected	: May 14, 2026, 09:49 AM	Report Date	: May 14, 2026, 12:30 PM.

NABL-M(EL)T-03192 MEDICAL ENTRY LEVEL TESTING LABORATORY RECOGNITION PROGRAM

Test Description	Value(s)	Unit(s)	Reference Range

Urine Routine and Microscopic Examination

Physical Examination			
Volume *	20	mL	-
Colour *	Pale yellow	-	Pale yellow
Transparency *	**Slightly Hazy H***	-	Clear
Deposit *	Absent	-	Absent
Chemical Examination			
Reaction (pH) *Double Indicator*	5.0	-	4.5 - 8.0
Specific Gravity *Ion Exchange*	**1.005 L***	-	1.010 - 1.030
Urine Glucose (sugar) *Oxidase / Peroxidase*	Negative	-	Negative
Urine Protein (Albumin) *Acid / Base Colour Excahnge*	Negative	-	Negative
Urine Ketones (Acetone) *Legals Test*	Negative	-	Negative
Blood *Peroxidase Hemoglobin*	Negative	-	Negative
Leucocyte esterase *Enzymatic Reaction*	Negative	-	Negative
Bilirubin Urine *Coupling Reaction*	Negative	-	Negative
Nitrite *Griless Test*	Negative	-	Negative
Urobilinogen *Ehrlichs Test*	Normal	-	Normal
Microscopic Examination			
Pus Cells (WBCs) *	**10-12 H***	/hpf	0 - 5
Epithelial Cells *	**6-7 H***	/hpf	0 - 4
Red blood Cells *	3-4	/hpf	Absent
Crystals *	Absent	-	Absent
Cast *	Absent	-	Absent
Yeast Cells *	Absent	-	Absent
Amorphous deposits *	Absent	-	Absent
Bacteria *	**Present(+) H***	-	Absent
Protozoa *	Absent	-	Absent

Interpretation:
URINALYSIS- Routine urine analysis assists in screening and diagnosis of various metabolic, urological, kidney and liver disorders.

Protein: Elevated proteins can be an early sign of kidney disease. Urinary protein excretion can also be temporarily elevated by strenuous exercise, orthostatic proteinuria, dehydration, urinary tract infections and acute illness with fever

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)
(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.

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Patient NAME	**: Ms Divya**		
DOB/Age/Gender	: 38 Y/Female	Report STATUS	: Final Report
Patient ID / UHID	: 16746904/RCL16605898	Barcode NO	: 32276138
Referred BY	: Self	Sample Type	: Spot Urine
Sample Collected	: May 14, 2026, 09:49 AM	Report Date	: May 14, 2026, 12:30 PM.

NABL-M(EL)T-03192
MEDICAL ENTRY LEVEL TESTING LABORATORY RECOGNITION PROGRAM

Test Description	Value(s)	Unit(s)	Reference Range

Glucose: Uncontrolled diabetes mellitus can lead to presence of glucose in urine. Other causes include pregnancy, hormonal disturbances, liver disease and certain medications.

Ketones: Uncontrolled diabetes mellitus can lead to presence of ketones in urine. Ketones can also be seen in starvation, frequent vomiting, pregnancy and strenuous exercise.

Blood: Occult blood can occur in urine as intact erythrocytes or haemoglobin, which can occur in various urological, nephrological and bleeding disorders.

Leukocytes: An increase in leukocytes is an indication of inflammation in urinary tract or kidneys. Most common cause is bacterial urinary tract infection.

Nitrite: Many bacteria give positive results when their number is high. Nitrite concentration during infection increases with length of time the urine specimen is retained in bladder prior to collection.

pH: The kidneys play an important role in maintaining acid base balance of the body. Conditions of the body producing acidosis/ alkalosis or ingestion of certain type of food can affect the pH of urine.

Specific gravity: Specific gravity gives an indication of how concentrated the urine is. Increased specific gravity is seen in conditions like dehydration, glycosuria and proteinuria while decreased specific gravity is seen in excessive fluid intake, renal failure and diabetes insipidus.

Bilirubin: In certain liver diseases such as biliary obstruction or hepatitis, bilirubin gets excreted in urine.

Urobilinogen: Positive results are seen in liver diseases like hepatitis and cirrhosis and in cases of haemolytic anaemia.

*** End Of Report ***

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)
(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.

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TERMS AND CONDITIONS

1. Laboratory reports are generated solely for informational and interpretational purposes for qualified medical practitioners and must be read in conjunction with the patient's clinical history, examination findings, and other diagnostic parameters. The laboratory does not provide medical advice, diagnosis, or treatment recommendations.
2. All the test results are based strictly on the specimen/sample and information received under the details provided at the time of registration. The patient/customer or referring entity is solely responsible for the accuracy and completeness of details furnished before sample submission. The laboratory shall not be liable for errors, misidentification, or incorrect results arising due to inaccurate, incomplete, or misleading information provided at the time of registration.
3. The patient/customers must disclose all relevant conditions including but not limited to fasting status, medications, dietary intake, alcohol consumption, lifestyle factors, ongoing treatments, and medical history that may affect test outcomes. The laboratory shall not be responsible for any deviation in results due to non-disclosure or incorrect disclosure of such information.
4. Test results are dependent on the quality, integrity, and timing of the specimen received. The laboratory reserves the right to reject or request repeat samples in cases of inadequate, hemolyzed, contaminated, or otherwise compromised samples. No liability shall arise for inability to process or delay due to such reasons.
5. Test results are subject to inherent biological variation and analytical variation depending on the nature of the test, methodology, instrumentation, and pre-analytical factors. While the laboratory follows established quality control and assurance standards, minor variations in results may occur and should be interpreted in conjunction with clinical findings, patient history, and other diagnostic information. The laboratory does not guarantee absolute accuracy, precision, or reproducibility of results across different testing instances or laboratories, and no liability shall arise on account of such inherent variations.
6. All turnaround times are indicative, not guaranteed. Delays may occur due to test complexity, sample condition, referral testing, or quality control procedures, logistics, or other operational constraints.
7. Reports may be released via digital or physical modes such as Email, SMS, WhatsApp, Mobile App, Web Portal, or Printed Copy, subject to completion of testing, quality validation, and payment clearance. Accessing the report through any digital medium shall constitute acceptance of electronic records.
8. All patient information and reports are treated as confidential and may be shared only with patient/customer, authorized healthcare professionals, regulatory bodies, or as required by law.
9. Laboratory reports are not valid for medico-legal purposes unless specifically requested, processed, and certified as such in accordance with applicable laws.
10. To the maximum extent permitted under applicable law, the laboratory shall not be liable for any direct, indirect, incidental, consequential, or special damages arising out of or in connection with the use of, reliance upon, or interpretation of the laboratory report. The laboratory shall have no liability for any clinical decisions, diagnosis, or treatment undertaken on the basis of the report. In all circumstances, the liability of the laboratory, if any, shall be strictly limited to the amount actually paid for the specific test(s) giving rise to such claim.
11. In case of critical, alarming, or unexpected test results, you are advised to contact the laboratory immediately for further guidance and thereafter consult a qualified medical practitioner without delay. Test results should always be interpreted in conjunction with the patient's clinical history and clinical findings.
12. Accessing reports through digital platforms constitutes acceptance of electronic delivery and record storage.
13. The laboratory shall not be liable for any delays in performance or failure to perform its obligations arising out or in connection with events beyond reasonable control, including but not limited to natural disasters, pandemics, acts of God, strikes, labor disputes, transportation disruptions, regulatory or governmental actions, technical failures, or any other unforeseen circumstances.
14. All disputes arising from laboratory services shall be subject to the jurisdiction of courts located in Delhi.